Clinical Guideline: Management of Hypertension in Adults

National Health Authority (Fictional) · Document NHA-CG-03 · Version 2.1 · Issued June 2026 · Synthetic Demonstration Edition

§1 Scope

Management of hypertension in adults, including patients with diabetes and chronic kidney disease.

§2 Blood pressure targets

- Office blood pressure below 140/90 mmHg is the minimum standard for all adults.
- A target below 130/80 mmHg is recommended for patients with diabetes, established cardiovascular disease, or CKD, if tolerated.
- Systolic pressure of 140 mmHg or above is recorded as uncontrolled and counts as an open care gap ('BP above target').

§3 Treatment

- Initiate lifestyle measures for all patients: sodium reduction, weight management, physical activity, and smoking cessation support.
- First-line drug classes are ACE inhibitors or ARBs, calcium channel blockers, and thiazide-like diuretics; combination therapy is preferred when baseline pressure is 20/10 mmHg or more above target.
- ACE inhibitors or ARBs are preferred in diabetes with albuminuria and in CKD.
- Review within 4 weeks of any treatment change until controlled; every 6 months thereafter.

§4 Resistant hypertension

- Confirm adherence and out-of-office readings before diagnosing resistance.
- Blood pressure uncontrolled on three agents including a diuretic warrants specialist referral.

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